



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رضوى عبدالله على ابراهيم

المحافظة : الجيزة الحي / المدينة : بولاق الدكرور العنوان : ++++++

رقم البطاقة : 28112301201182 رقم التليفون : 01005026389 تاريخ تسجيل الإستمارة : 03-10-2026

العمر (تاريخ الميلاد) : 30-12-1981 المهنة : موظفة

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور قسم العلاج : الاورام [14163]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رضوى عبدالله على ابراهيم
National ID: 28112301201182



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



Name: رضوى عبدالله علي ابراهيم		GUSTAVE ROUSSY	
MR. : 8228			
Detailed Medical Report			
			السيد
تحية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient has a history of metastatic breast cancer to bone since 2022 and has been receiving endocrine therapy with FASLODEX (fulvestrant) and IBRANCE (palbociclib) since the initiation of FASLODEX 500 mg and IBRANCE 125 mg on a 1 × 1 × 21 schedule. The most recent endocrine therapy regimen included ZOLADEX (goserelin) and ZOMETA (zoledronic acid) administered on day 13/12/2025, with the last ZOLADEX dose recorded at 10.8 mg and the last ZOMETA dose also at 13/12/2025. Laboratory results at that time showed a total leukocyte count (TLC) of $3.9 \times 10^9/L$ and neutrophils at $2.44 \times 10^9/L$. On 05/04/2026 the patient continued the same endocrine regimen (FASLODEX, IBRANCE, ZOLADEX) and received ZOMETA on day 05/2026. Day-care pharmacy services were utilized for medication administration, and laboratory work was accepted and processed without noted abnormalities. On 06/06/2026 the oncology team documented the patient's metastatic breast cancer as bone-predominant, estrogen receptor-positive, HER2-negative, with a PI3KCA mutation identified in 2020. The patient's prior treatment history includes right-treatment breast cancer with radiation, tamoxifen, and ZOLADEX, followed in 2022 by bone metastases treated with ZOLADEX, FASLODEX, and KISQALI, and then IBRANCE initiated in 05/2023. A PET-CT performed on 04/2026 revealed a left seventh rib lesion with an SUV of 3.5 and thyroiditis with an SUV of 7 on neck ultrasound; thyroid function tests were pending. Current endocrine therapy includes ZOLADEX, ZOMETA, FASLODEX, and IBRANCE 125 mg on a 1 × 1 × 21 schedule. On 05/07/2026 the patient was noted to have a PIK3CA mutation, rendering her eligible for PI3K pathway-targeted therapy with Truqap, pending NGO approval. The patient continued FASLODEX and IBRANCE 125 mg on the same schedule. On 06/09/2026 the patient received FASLODEX, IBRANCE 125 mg, ZOLADEX, and supportive therapy. Laboratory results were within normal limits, and a bone scan was pending.			
تحريراً في ,			
		التاريخ : 20-09-2026	الوقت :
		علياء احمد أبو النصر شهاب الدين	
		الطبيب المعالج :	
يعتمد ,			



Imaging report

Pat. Name: Radwa ALSISI	Pat. ID: R2215010
Procedure Name: PET - CT - Radiology	Date: 26/04/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- o 44 years old patient with managed metastatic right breast cancer .
- o PET/CT requested for assessment.

Technique:

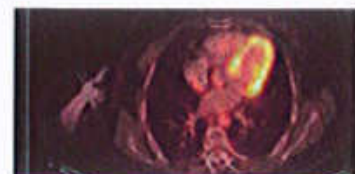
- o For the forementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.
- o Hepatic Reference activity ~3.3.
- o IV contrast was injected.



Report

No previous PET-CT available for comparison.

Evidence of right mastectomy with no sizable FDG avid neoplastic lesions could be detected.





Imaging report

Pat. Name: Radwa ALSISI

Pat. ID: R2215010

FDG avid left 7th rib sclerotic lesion of SUVmax~3.5.



Evidence of right humeral shaft and dorsal vertebrae fixation.

Non FDG avid left femoral head sclerotic lesion.



Non FDG avid LV4 lytic lesion with sclerotic margins.



Head and Neck:

- * Diffuse FDG uptake is noted over both thyroid lobes of SUVmax~7.5 likely due to chronic thyroiditis for correlation with thyroid hormonal profile, and antibodies.



- * No hypermetabolic cervical lymphadenopathy could be detected.
- * Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- * The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.



Imaging report

Pat. Name: Radwa ALSISI

Pat. ID: R2215010

Chest:

- * No hypermetabolic axillary or mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses.
- * There are no active pleural abnormalities.
- * Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected.
- * Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- * The pancreas shows normal FDG avidity.
- * There are no active adrenal nodules.
- * Physiologic FDG excretion is seen in the kidneys and bladder.

Prof. Mohamed Khalil MD

Prof. Doria Salem MD
Prof. Ashraf Selim MD

Prof. Mohamed El Ghawaby MD
Prof. Nabil Awad MD

Prof. Hussein Attia MD
Prof. Nadia Hassan MD

Prof. Aziz Mordbour MD
Prof. Sahar Nassef MD

Prof. Hussein Abou El Motassim MD
Prof. Alaa El Ibrahimy MD



Imaging report

Pat. Name: Radwa ALSISI

Pat. ID: R2215010

Conclusion:

PET-CT study for a known case of managed metastatic right breast cancer shows:

- *Evidence of right mastectomy with clear operative bed and NO FDG avid local tumor recurrence or regional LNs.*
- *Mildly active FDG avid left 7th rib sclerotic metastatic deposit.*
- *Non FDG avid left femoral head sclerotic lesion likely old healed lesion.*
- *NO other definite FDG avid metastases detected.*

Thank you for referral

Dina Thabet, MSc

Ahmed Zaher , MD

Nuclear medicine specialist

Professor of nuclear medicine

National Cancer Institute.

National Cancer Institute

For any inquiries regarding the findings in the report please contact us on

radreport@cairoscan.com.eg



Patient Name	: الأستاذ / رضوى عبدالله على ابراهيم	Request No.	: 13007413
Patient No.	: 28112301201182	Request Date	: 13/04/2023
Path No.	: SR-482/23	Verification Date	: 14/05/2023
Sex	: Female	Financial Category	: اورام خارج السلام - نفقة الدولة
Age	: 41 Y.	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical data: Right breast mass, (post neoadjuvant therapy)

Nature of specimen: Slide revision

Gross examination

6 blocks (49851) + 11 slides

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Multiple foci of invasive duct carcinoma .
- Grade (differentiation+ nuclear pleomorphism + mitosis); GII (2+2+2)
- Skin involvement: Free
- DCIS: Focal
- Angiolinvasion: Focal
- Lymph node involvement: yes (1/9).
- Resection Margins: Free.
- Histologic response to therapy: Partial pathological response.
- Additional pathologic findings: Poorly fixed blocks, Hyalinosis, Fibrosis in surrounding breast.

ER: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8)

PR: Moderate nuclear reaction in 60% of tumor cells (Allred score 6/8)

Her-2: No membrane reaction of tumor cells (score 0)

Ki 67: 10 % nuclear reaction

Diagnosis:

Right breast mass, slide revision :

Invasive duct carcinoma, Grade II

Verified by :

Dr.Marwa mahmoud

Patient Name : الأستاذ / رضوى عبداللّه على ابراهيم
Patient No. : 28112301201182
Path No. : SR-482/23
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13007413
Request Date : 13/04/2023
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Financial Category : اورام خارج السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Partial Pathological response to therapy.
- ✓ Focal DCIS (15%)
 - ✓ Positive Lymph nodes (1/8)
 - ✓ ER : Positive (7/8)
 - ✓ PR : Positive (6/8)
 - ✓ HER2 : Negative (score 0)
 - ✓ Ki67: 15%

Prof.Gamal Abdelatty
Dr.Marwa Mahmoud

Verified by :
Dr.Marwa mahmoud

Printing Date : 15/05/2023 09:32:10

Page 2 of 2

Printing By : Rana fathy

اسم الفرع ميدان لبنان
 مدير الفرع د. بهاء الدين جاد
 مدير الوحدة ا.د. عبد العزيز النقي
 طبيب الوحدة ا.د. علا الميسى
 ا.د. ايات ابراهيم

المختبر
 al mokhtabar
 moamena kamel lab
 معامل مؤمنة كامل



رئيس مجلس الإدارة
 ا.د. مؤمنة كامل

مدير المعامل
 ا.د. هند الشربيني

PATIENT NAME رضوى عبد الله الميسى			Registered 10-04-2026 13:01:16	Collected 10-04-2026 13:02:25
			Authenticated 10-04-2026 19:02:05	Printed 11-04-2026 09:03:04
Visit Number 27026502185	Age 45 Year	Gender Female	Referred By Prof: -	Client ID 474

Test Name	Result	Unit	Reference Range	Previous Result
Diabetic Profile				
Fasting Blood Glucose	79	mg/dL	70 - 110	
Kidney Function Tests				
Ionized Calcium	1.16	mmol/L	1.12 - 1.32	1.18
Serum Creatinine	↑ 1.12	mg/dL	0.5 - 1.1	0.82
Serum Calcium	9.1	mg/dL	8.6 - 10.2	9.4



A. Nokaly
 Dr.ABD El-Aziz El-Nokaly
 Professor of Medical Bio-Chemistry,
 Faculty of Medicine, Al Azhar University

1-1

اسم الفرع ميدان لبنان
مدير الفرع د. بهاء الدين جاد
مدير الوحدة ا.د. هالة شبيب
طبيب الوحدة د. فائق رمضان
د. دعاء خميس

المختبر
al mokhtabar
moamena kamel lab
معامل مؤمنة كامل



رئيس مجلس الإدارة
ا.د. مؤمنة كامل

مدير المعامل
ا.د. هند الشربيني

PATIENT NAME رضوى عبد الله السيسى			Registered	10-04-2026 13:01:16	Collected	10-04-2026 13:02:25
Visit Number			Authenticated	10-04-2026 19:09:21	Printed	11-04-2026 09:03:05
27026502185	Age	Gender	Referred By		Client ID	
45 Year	Female		Prof : -		474	

HEMATOLOGY REPORT

Test Name	Result	Unit	Reference Range
Complete Blood Picture			
Haemoglobin (EDTA Blood)	12.4	g/dL	11.5 - 15.5
Haematocrit (PCV)	37.7	%	36 - 45
RBCs Count (EDTA Blood)	4.36	Millions / cmm	4 - 5.2
MCV	86.5	fl	80 - 100
MCH	28.4	pg	27 - 33
MCHC	32.9	g/dL	31 - 37
RDW-CV	15.6	%	11.5 - 15
Platelet Count	249	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	4.0	Thousands / cmm	4 - 11
Percent Values			
Neutrophils	63.9	%	2.56 x10 ⁹ /L
Lymphocytes	30.4	%	1.22 x10 ⁹ /L
Monocytes	5.7	%	0.23 x10 ⁹ /L
Eosinophils	0.0	%	0.00 x10 ⁹ /L
Basophils	0.0	%	0.00 x10 ⁹ /L
Absolute Values			
Neutrophils	2.56	x10 ⁹ /L	2 - 7
Lymphocytes	1.22	x10 ⁹ /L	1 - 4.8
Monocytes	0.23	x10 ⁹ /L	0.2 - 1
Eosinophils	0.00	x10 ⁹ /L	0.1 - 0.45
Basophils	0.00	x10 ⁹ /L	0 - 0.1
Other Cells			
Comment:			
RBCs show mild anisocytosis.			
Follow up is recommended.			



Hala Sheba
Dr. Hala Sheba
Professor of clinical pathology,
Faculty of Medicine, Cairo university

1-1



جمهورية مصر العربية
وزارة الداخلية
مطابق نظام المصالحات

رضوى
عبدالله على ابراهيم السيسى
١١ ش مصطفى مواتر النيل - القاهرة
المعوزة - الجيزة

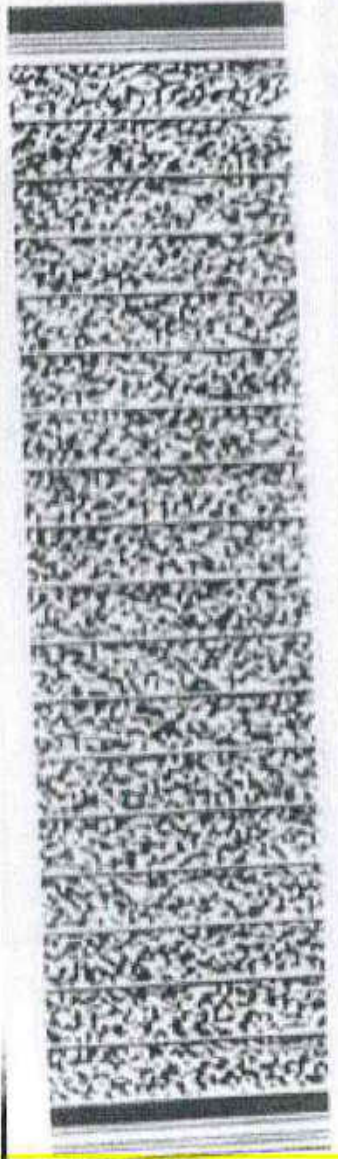
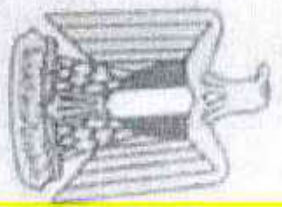
١٩٨٧/١٢/٣١

٢٨١ ١٢٣٠ ١٢٠ ١١٨٢

LG7557287



٢٠٢٥/٠٧
٢٨١١٢٣٠١٢٠١١٨٢
نائب رئيس تحرير
دار الجمهورية للصحافة
مستقلة
انتش
ياسر على محمود خلف
البطاقة سارية حتى ٢٠٢٢/٠٧/٢٩



Dr. Ali Ibrahim

Date : 13-09-2026 06:30

Medical No. : 460187

Prof : Ahmed El Bastawisy

Ins. Company : شركة التأمين

Osseous:

- Metabolic regression of the priorly seen low grade FDG-avid sclerotic osseous lesion at the lateral reflection of left 7th rib, currently eliciting SUVmax ~ 1.7 compared to SUVmax ~ 3.5.
- Stable non-metabolically active sclerotic osseous lesion at the head of left femur.



- Stable tracer-starved lucent osseous lesion with marginal sclerosis at the left aspect of LV4 body; likely benign.
- Still noted internal fixation of the proximal portion of right humerus.
- Still noted DV6 wedging fracture with bridging internal fixation with laminectomy of DV5 and DV7.
 - No other significant FDG-avid focal lesions or pathological lymph nodes to account for distant metastatic deposits all over the rest of the surveyed body.

V) Rest of the body:

- No apparent brain focal lesions or pathologic tracer distribution.
- Still noted enlarged thyroid gland with regression of the diffuse parenchymal metabolic activity, currently eliciting SUVmax ~ 4.7 compared to SUVmax ~ 7.5 with no gross nodularity detected; could be thyroiditis, for clinico-laboratory and U/S correlation.
- No FDG-avid pulmonary nodules, masses or infiltrates.
- No pleural or pericardial effusion, thickening or masses.

Odallah Ali Ibrahim

Date : 13-09-2026 06:30

Medical No. : 460187

تقنية المصطفين
Ins. Company :

Acian Prof: Ahmed El Bastawisy

- o Enlarged fatty liver with no FDG-avid focal lesions. No intra or extra-hepatic biliary radicle dilatation.
- o Normal appearance of the spleen, pancreas, kidneys, suprarenals and pelvic structures.
- o No pelvi-abdominal ascites or fluid collections.

VI) Side findings:

- o Left maxillary sinus mucosal disease.
- o Degenerative changes are noted over the spine.

VII) Normal physiological tracer bio-distribution is seen over the brain, rest of bone marrow, glandular structures, heart, bowel and urinary tract.

Conclusion:

Follow-up whole body (FDG) PET/CT study shows:

- Maintained clear right mastectomy bed of any metabolically active lesions to account for neoplastic recurrence.
- Metabolic regression of the left 7th rib sclerotic osseous lesion.
- Stable non-metabolically active sclerotic osseous lesion at the head of left femur.
- No other metabolically active lesions could be seen all over the rest of the surveyed body.

As Regards,

Dr. Moumen Katamesh, MSc
Nuclear Medicine

Dr. Yasser Mohamed
Professor of Nuclear Medicine

Name : Radya Abdallah Ali Ibrahim
Branch : Dokki
Ref.Physician Prof : Ahmed El Bastawisy

Date : 13-09-2026 06:30
Medical No. : 460187
شركة الشركة : Ins. Company

Follow-up Whole Body (18F-FDG) PET/CT Study

History and Clinical Data:

Patient data:

o Weight: 90 Kg, Blood Glucose level: 81 mg/dl.

Clinical background:

o 45-year-old patient with history of managed right breast cancer.

Radiopharmaceutical:

o 144.6 MBq of 18F-FDG was injected I.V.

Purpose of the study:

o Follow-up in comparison to the prior (FDG) PET/CT study report dated 26/04/2026.

Technique:

- o Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- o **Contrast Enhanced CT** images done for attenuation correction and anatomic localization were obtained followed by PET images from the vertex of the skull to the thighs.
- o PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- o The Standardized Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.
- o Reference Hepatic SUV max ~ 4.5.

Findings:

I) Right breast:

- o Maintained clear right mastectomy bed of any FDG-avid focal lesions to account for neoplastic recurrence.

II) Left breast:

- o No FDG-avid focal lesions could be seen within the left breast.

III) Mammary-related lymph nodes:

- o No significant FDG-avid or pathologically enlarged either axillary or other mammary-related lymph nodes.

IV) Distant: